

Pemetrex- Venus

(Pemetrexed Powder for concentrate for solution for infusion 100mg)

COMPOSITION

Each Vial contains Pemetrexed powder of 100mg, Mannitol of 106mg, Hydrochloric acid and Sodium Hydrochloride may have been added to adjust pH.

DOSAGE FORM

Parenteral. Lyophilised powder for concentrate for solution for infusion.

PRODUCT DESCRIPTION

Parenteral. Lyophilised powder for concentrate for solution for infusion.

A white to light yellow or green-yellow lyophilized powder packed in a 10ml type 1 clear tubular glass vial with stopper (Grey butyl rubber with an aluminium flip-off cap black colour).

Each Vial must be reconstituted with 4.2ml of 0.9% sodium chloride injection resulting in 25mg/ml solution. The reconstituted Pemetrexed solution must be further diluted into a solution of 0.9% Sodium Chloride Injection, so that the total volume of solution is 100ml. The solution for both reconstitution and dilution are formed as Clear colourless to yellow or green-yellow solution.

PHARMACODYNAMICS

ATC code: L01BA04

Pemetrexed is a multi-cancer antifolate agent that exerts its action by disrupting crucial folate-dependent metabolic processes essential for cell replication.

In vitro studies have shown that pemetrexed behave as a multitargeted antifolate by inhibiting thymidylate synthase (TS), dihydrofolate reductase (DHFR), and glycinamide ribonucleotide formyl transferase (GARFT), which are key folate-dependent enzymes for the de novo biosynthesis of thymidine and purine nucleotides. Pemetrexed is transported into cells by both the reduced folate carrier and membrane folate binding protein transport systems. Once in the cell, pemetrexed is rapidly and efficiently converted to polyglutamate forms by the enzyme folylpolyglutamate synthetase. The polyglutamate forms are retained in cells and are even more potent inhibitors of TS and GARFT. Polyglutamation is a time and concentration dependent process that occurs in tumor cells and to lesser extent, in normal tissues. Polyglutamated metabolites have an increased intracellular half-life resulting in prolonged drug action in malignant cells.

PHARMACOKINETICS

The pharmacokinetic properties of pemetrexed following single-agent administration have been evaluated in 426 cancer patients with a variety of solid tumours at doses ranging from 0.2 to 838 mg/m² infused over a 10-minute period.

Pemetrexed has a steady-state volume of distribution of 9 l/m². In vitro studies indicate that pemetrexed is approximately 81 % bound to plasma proteins. Binding was not notably affected by varying degrees of renal impairment. Pemetrexed undergoes limited hepatic metabolism. Pemetrexed is primarily eliminated in the urine, with 70 % to 90 % of the administered dose

being recovered unchanged in urine within the first 24 hours following administration. In vitro studies indicate that pemetrexed is actively secreted by OAT3 (organic anion transporter). Pemetrexed total systemic clearance is 91.8 ml/min and the elimination half-life from plasma is 3.5 hours in patients with normal renal function (creatinine clearance of 90 ml/min). Between patient variability in clearance is moderate at 19.3 %. Pemetrexed total systemic exposure (AUC) and maximum plasma concentration increase proportionally with dose. The pharmacokinetics of pemetrexed are consistent over multiple treatment cycles.

The pharmacokinetic properties of pemetrexed are not influenced by concurrently administered cisplatin. Oral folic acid and intramuscular Vitamin B12 supplementation do not affect the pharmacokinetics of pemetrexed.

INDICATION

Malignant pleural mesothelioma

Pemetrex- Venus in combination with cisplatin is indicated for the treatment of chemotherapy naïve patients with unresectable malignant pleural mesothelioma.

Non-small Cell Lung Cancer:

Pemetrex- Venus in combination with cisplatin is indicated for the first-line treatment of patients with locally advanced or metastatic non-small cell lung cancer (NSCLC) other than predominantly squamous cell histology.

Pemetrex- Venus is indicated as monotherapy for the maintenance treatment of locally advanced or metastatic non-small cell lung cancer other than predominantly squamous cell histology in patients whose disease has not progressed immediately following platinum-based chemotherapy.

Pemetrex- Venus is indicated as monotherapy for the second-line treatment of patients with locally advanced or metastatic non- small cell lung cancer other than predominantly squamous cell histology.

Pemetrex- Venus (Pemetrexed Powder for concentrate for solution 100mg) in combination with pembrolizumab and platinum chemotherapy, is indicated for the first line treatment of patients with metastatic non-squamous NSCLC, with no EGFR or ALK genomic tumour aberrations.

RECOMMENDED DOSE

Posology

Pemetrexed must only be administered under the supervision of a physician qualified in the use of anti-cancer chemotherapy.

Pemetrexed - Venus in combination with cisplatin

The recommended dose of Pemetrex- Venus is 500 mg/m² of body surface area (BSA) administered as an intravenous infusion over 10 minutes on the first day of each 21-day cycle. The recommended dose of cisplatin is 75 mg/m² BSA infused over two hours approximately 30 minutes after completion of the pemetrexed infusion on the first day of each 21-day cycle.

Patients must receive adequate anti-emetic treatment and appropriate hydration prior to and/or after receiving cisplatin.

Pemetrexed as single agent

Pemetrexed Venus in combination with pembrolizumab

The recommended dose of Pemetrexed Venus when administered with pembrolizumab and platinum chemotherapy for the initial treatment of metastatic non-squamous NSCLC in patients with a creatinine clearance (calculated by Cockcroft-Gault equation) of 45 mL/min or greater is 500 mg/m² as an intravenous infusion over 10 minutes administered after pembrolizumab and prior to carboplatin or cisplatin on Day 1 of each 21-day cycle for 4 cycles. Following completion of platinum-based therapy, treatment with Pemetrexed Venus with or without pembrolizumab is administered until disease progression or unacceptable toxicity (see also pembrolizumab product insert for specific dosing advice).

Pre-medication regimen

To reduce the incidence and severity of skin reactions, a corticosteroid should be given the day prior to, on the day of, and the day after pemetrexed administration. The corticosteroid should be equivalent to 4 mg of dexamethasone administered orally twice a day.

To reduce toxicity, patients treated with pemetrexed must also receive vitamin supplementation. Patients must take oral folic acid or a multivitamin containing folic acid (350 to 1000 micrograms) on a daily basis. At least five doses of folic acid must be taken during the seven days preceding the first dose of pemetrexed, and dosing must continue during the full course of therapy and for 21 days after the last dose of pemetrexed. Patients must also receive an intramuscular injection of Vitamin B12 (1000 micrograms) in the week preceding the first dose of pemetrexed and once every three cycles thereafter. Subsequent Vitamin B12 injections may be given on the same day as pemetrexed.

Monitoring

Patients receiving pemetrexed should be monitored before each dose with a complete blood count, including a differential white cell count (WCC) and platelet count. Prior to each chemotherapy administration blood chemistry tests should be collected to evaluate renal and hepatic function. Before the start of any cycle of chemotherapy, patients are required to have the following: absolute neutrophil count (ANC) should be ≥ 1500 cells/mm³ and platelets should be $\geq 100,000$ cells/mm³.

Creatinine clearance should be ≥ 45 ml/min.

The total bilirubin should be ≤ 1.5 times upper limit of normal. Alkaline phosphatase (AP), aspartate aminotransferase (AST or SGOT) and alanine aminotransferase (ALT or SGPT) should be ≤ 3 times upper limit of normal. Alkaline phosphatase, AST and ALT ≤ 5 times upper limit of normal is acceptable if liver has tumour involvement.

Dose adjustments

Dose adjustments at the start of a subsequent cycle should be based on nadir hematologic counts or maximum nonhematologic toxicity from the preceding cycle of therapy. Treatment may be delayed to allow sufficient time for recovery. Upon recovery patients should be retreated using the guidelines in Tables 1, 2 and 3, which are applicable for pemetrexed used as a single agent or in combination with cisplatin.

Table 1 - Dose modification table for Pemetrexed (as single agent or in combination) and cisplatin – Haematologic toxicities	
Nadir ANC < 500 /mm ³ and nadir platelets ≥ 50,000/mm ³	75 % of previous dose (both pemetrexed and cisplatin)
Nadir platelets < 50,000 /mm ³ regardless of nadir ANC	75 % of previous dose (both pemetrexed and cisplatin)
Nadir platelets < 50,000 /mm ³ with bleeding ^a , regardless of nadir ANC	50 % of previous dose (both pemetrexed and cisplatin)

^a These criteria meet the National Cancer Institute Common Toxicity Criteria (CTC v2.0; NCI 1998) definition of ≥CTC Grade 2 bleeding.

If patients develop non-hematologic toxicities ≥ Grade 3 (excluding neurotoxicity), pemetrexed should be withheld until resolution to less than or equal to the patient's pre therapy value.

Treatment should be resumed according to the guidelines in Table 2.

Table 2- Dose modification table for Pemetrexed (as single agent or in combination) and cisplatin – Non-hematologic toxicities ^{a,b}		
	Dose of Pemetrexed (mg/m ³)	Dose for cisplatin (mg/m ²)
Any Grade 3 or 4 toxicities except mucositis	75% of previous dose	75% of previous dose
Any diarrhea requiring hospitalization (irrespective of grade) or grade 3 or 4 diarrhea	75% of previous dose	75% of previous dose
Grade 3 or 4 mucositis	50% of previous dose	100% of previous dose

^a National Cancer Institute Common Toxicity Criteria (CTVv2.0; NCI 1998)

^b Excluding neurotoxicity

In the event of neurotoxicity, the recommended dose adjustment for Pemetrexed and cisplatin is documented in Table 3. Patient should discontinue therapy if Grade 3 or 4 neurotoxicity is observed.

Table 3 – Dose modification for Pemetrexed (as single agent or in combination) and cisplatin- Neurotoxicity		
CTC* Grade	Dose of Pemetrexed (mg/m ²)	Dose for cisplatin (mg/m ²)
0-1	100 % of previous dose	100 % of previous dose
2	100 % of previous dose	50 % of previous dose

^a National Cancer Institution Common Toxicity Criteria (CTC v2.0; VCI 1998)

Treatment with Pemetrexed should be discontinued if a patient experiences any hematologic or non-hematologic Grade 3 or 4 toxicity after 2 dose reductions or immediately if Grade 3 or 4 neurotoxicity is observed.

Elderly

In clinical studies, there has been no indication that patients 65 years of age or older are at increased risk of adverse events compared to patients younger than 65 years old. No dose

reductions other than those recommended for all patients are necessary.

Paediatric population

Pemetrex- Venus not recommended for use in patients under 18 years of age, as safety and efficacy have not been established in this group of patients..

Patients with renal impairment (Standard Cockcroft and Gault formula or Glomerular Filtration Rate measured Tc99m-DPTA serum clearance method)

Pemetrexed is primarily eliminated unchanged by renal excretion. Patients with creatinine clearance of ≥ 45 ml/min required no dose adjustments other than those recommended for all patients. There are insufficient data on the use of pemetrexed in patients with creatinine clearance below 45 ml/min; therefore, the use of pemetrexed is not recommended.

Patients with hepatic impairment

No relationships between AST (SGOT), ALT (SGPT), or total bilirubin and pemetrexed pharmacokinetics were identified. However patients with hepatic impairment such as bilirubin > 1.5 times the upper limit of normal and/or aminotransferase > 3.0 times the upper limit of normal (hepatic metastases absent) or > 5.0 times the upper limit of normal (hepatic metastases present) have not been specifically studied.

ROUTE OF ADMINISTRATION

Intravenous infusion.

CONTRAINDICATION

- Hypersensitivity to the active substances or any of the excipients listed.
- Breast – feeding.
- Concomitant yellow fever vaccine

WARNING AND PRECAUTIONS

Pemetrexed can suppress bone marrow function as manifested by neutropenia, thrombocytopenia, and anaemia (or pancytopenia) (see section 4.8). Myelosuppression is usually the dose-limiting toxicity. Patients should be monitored for myelosuppression during therapy and pemetrexed should not be given to patients until absolute neutrophil count (ANC) returns to $\geq 1,500$ cells/mm³ and platelet count returns to $\geq 100,000$ cells/mm³. Dose reductions for subsequent cycles are based on nadir ANC, platelet count, and maximum non-haematologic toxicity seen from the previous cycle.

All patients treated with pemetrexed must be instructed to take folic acid and vitamin B12 as a prophylactic measure to reduce treatment-related toxicity.

Skin reactions have been reported in patients not pre-treated with a corticosteroid. Pre-treatment with dexamethasone (or equivalent) can reduce the incidence and severity of skin reactions.

The use of pemetrexed in patients with creatinine clearance of <45 ml/min is not recommended.

Patients with mild to moderate renal insufficiency (creatinine clearance from 45 to 79ml/min)

should avoid taking non-steroidal anti-inflammatory drugs (NSAIDs), such as ibuprofen, and acetylsalicylic acid (>1.3g daily) for 2 days before, on the day of, and 2 days following pemetrexed administration.

In patients with mild to moderate renal insufficiency eligible for pemetrexed therapy, NSAIDs with long elimination half-lives should be interrupted for at least 5 days prior to, on the day of, and at least 2 days following pemetrexed administration

Serious renal events, including acute renal failure, have been reported with pemetrexed alone or in association with other chemotherapeutic agents. Many of the patients in whom these occurred had underlying risk factors for the development of renal events, including dehydration or pre-existing hypertension or diabetes.

The effect of third-space fluid, such as pleural effusion or ascites, on pemetrexed is not fully defined. Thus, drainage of third-space fluid collection prior to pemetrexed treatment should be considered but may not be necessary.

Due to the gastrointestinal toxicity of pemetrexed given in combination with cisplatin, severe dehydration has been observed. Therefore, patients should receive adequate anti-emetic treatment and appropriate hydration prior to and/or after receiving treatment.

Serious cardiovascular events, including myocardial infarction and cerebrovascular events, have been uncommonly reported with pemetrexed, usually when given in combination with another cytotoxic agent. Most of the patients in whom these events have been observed had pre-existing cardiovascular risk factors.

Immunodepressed status is common in cancer patients. As a result, concomitant use of live attenuated vaccines is not recommended.

Pemetrexed can have genetically damaging effects. Sexually mature males are advised not to father a child during the treatment and up to 6 months thereafter. Contraceptive measures or abstinence are recommended. Owing to the possibility of pemetrexed treatment causing irreversible infertility, men are advised to seek counselling on sperm storage before starting treatment.

Women of childbearing potential must use effective contraception during treatment with pemetrexed.

Cases of radiation pneumonitis have been reported in patients treated with radiation either prior, during, or subsequent to their pemetrexed therapy. Particular attention should be paid to these patients, and caution exercised with use of other radio-sensitising agents.

Special Warning and precautions.

Nephrogenic diabetes insipidus and renal tubular necrosis were also reported in post-marketing setting with pemetrexed alone or with other chemotherapeutic agents. Most of these events resolved after pemetrexed withdrawal. Patients should be regularly monitored for acute tubular necrosis, decreased renal function and signs and symptoms of nephrogenic diabetes insipidus (e.g. hypernatremia).

INTERACTIONS WITH OTHER MEDICAMENTS

Non-Steroidal Anti-Inflammatory Drugs (NSAIDs)

Although ibuprofen (400mg four times a day) can decrease the clearance of pemetrexed, it can be administered with Pemetrexed in patients with normal renal function (creatinine clearance $\geq 80\text{mL/min}$). No dose adjustment of Pemetrexed is needed with concomitant NSAIDs in patients with normal renal function.

Caution should be used when administering NSAIDs concurrently with Pemetrexed to patients with mild to moderate renal insufficiency (creatinine clearance from 45 to 79 mL/min).

NSAIDs with short elimination half-lives (e.g., diclofenac, indomethacin) should be avoided for a period of 2 days before, the day of and 2 days following administration of Pemetrexed. In the absence of data regarding potential interaction between Pemetrexed and NSAIDs with longer half-lives (e.g. meloxicam naburnetone), patients taking these NSAIDs should interrupt dosing for at least 5 days before, the day of and 2 days following Pemetrexed administration. If concomitant administration of NSAIDs is necessary, patients should be monitored closely for toxicity, especially myelosuppression, renal and gastrointestinal toxicity.

Nephrotoxic Drugs

Pemetrexed is primarily eliminated unchanged renally as a result of glomerular filtration and tubular secretion.

Concomitant administration of nephrotoxic drugs could result in delayed clearance of Pemetrexed. Concomitant administration of substances that are also tubularly secreted (e.g., Probenecid) could potentially result in delayed clearance of pemetrexed.

PREGNANCY AND LACTATION

Teratogenic Effects

Pregnancy Category D. Based on its mechanism of action, Pemetrexed can cause fetal harm when administered to a pregnant woman. There are no adequate and well controlled studies of Pemetrexed in pregnant women. Pemetrexed was embryotoxic and teratogenic in mice. If Pemetrexed is used during pregnancy or if the patient becomes pregnancy while taking this drug, the patients should be apprised of the potential hazards to the fetus.

Women of childbearing potential should be advised to use effective contraceptive measures to prevent pregnancy during the treatment with Pemetrexed.

Nursing mothers.

It not known whether Pemetrexed or its metabolites are excreted in human milk. Because many drugs are excreted in human milk, and because of the potential for serious adverse reactions in nursing infants from Pemetrexed, a decision should be made to discontinue nursing or discontinue the drug, taking into account the importance of the drug from mother.

SIDE EFFECTS

Summary of the safety profile

The most commonly reported undesirable effects related to pemetrexed, whether used as monotherapy or in combination, are bone marrow suppression manifested as anaemia, neutropenia, leukopenia, thrombocytopenia; and gastrointestinal toxicities, manifested as anorexia, nausea, vomiting, diarrhoea, constipation, pharyngitis, mucositis, and stomatitis. Other undesirable effects include renal toxicities, increased aminotransferases, alopecia, fatigue, dehydration, rash, infection/sepsis and neuropathy. Rarely seen events include Stevens-Johnson syndrome and Toxic epidermal necrolysis.

Tabulated list of adverse reaction.

The table lists the adverse drug events regardless of causality associated with pemetrexed used either as a monotherapy treatment or in combination with cisplatin from the pivotal registration studies and from the post marketing period.

Table: Frequencies of all grades adverse drug events regardless of causality from the pivotal registration studies:

System Organ class	Very common	Common	Uncommon	Rare	Very rare	Not known
Infections and infestations	Infection Pharyngitis	Sepsis ^b			Dermo-hypodermis	
Blood and lymphatic system disorders	Neutropenia Leukopenia Hemoglobin decreased	Febrile neutropenia Platelet count decreased	Pancytopenia	Autoimmune hemolytic anemia		
Immune System disorders		Hypersensitivity		Anaphylactic shock		
Metabolism and nutrition disorders		Dehydration				
Nervous system disorders		Taste disorder Peripheral motor neuropathy Peripheral sensory neuropathy Dizziness	Cerebrovascular accident Ischaemic stroke Haemorrhage intracranial			
Eye disorders		Conjunctivitis Dry eye Lacrimation increased Keratoconjunctivitis sicca Eyelid oedema				

		Ocular surface disease				
Cardiac disorders		Cardiac failure Arrhythmia	Angina Myocardial infarction Coronary artery disease Arrhythmia supraventricular			
Vascular disorders			Peripheral ischaemia ^c			
Respiratory, thoracic and mediastinal disorders			Pulmonary embolism Interstitial pneumonitis ^{bd}			
Gastrointestinal disorders	Stomatitis Anorexia Vomiting Diarrhoea Nausea	Dyspepsia Constipation Abdominal pain	Rectal haemorrhage Gastrointestinal haemorrhage Intestinal perforation Oesophagitis Colitis ^e			
Hepatobiliary disorders		Alanine aminotransferase increased Aspartate aminotransferase increased		Hepatitis		
Skin and subcutaneous tissue disorders	Rash Skin exfoliation	Hyperpigmentation Pruritus Erythema multiforme Alopecia Urticaria		Erythema	Stevens-Johnson syndrome ^b Toxic epidermal necrolysis ^b Pemphigoid Dermatitis bullous Acquired epidermolysis bullosa Erythematous oedema ^f Pseudocellulitis Dermatitis	

					Eczema Prurigo	
Renal and urinary disorders	Creatinine clearance decreased Blood creatinine increased ^e	Renal failure Glomerular filtration rate decreased				Nephrogenic diabetes insipidus Renal tubular necrosis
General disorders and administration site conditions	Fatigue	Pyrexia Pain Oedema Chest pain Mucosal inflammation				
Investigations		Gamma-glutamyltransferase increased				
Injury, poisoning and procedural complications			Radiation oesophagitis Radiation pneumonitis	Recall phenomenon		

^a with and without neutropenia

^b in some cases fatal

^c sometimes leading to extremity necrosis

^d with respiratory insufficiency

^e seen only in combination with cisplatin

^f mainly of the lower limbs

SYMPTOMS AND TREATMENT OF OVERDOSE

Reported symptoms of overdose include neutropenia, anaemia, thrombocytopenia, mucositis, sensory polyneuropathy, and rash. Anticipated complications of overdose include bone marrow suppression as manifested by neutropenia, thrombocytopenia, and anaemia. In addition, infection with or without fever, diarrhoea, and/or mucositis may be seen. In the event of suspected overdose, patients should be monitored with blood counts and should receive supportive therapy as necessary. The use of calcium folinate / folinic acid in the management of pemetrexed overdose should be considered.

EFFECT ON ABILITY TO DRIVE AND USE MACHINE

Please incorporate this below mentioned Details No studies on the effects on the ability to drive and use machines have been performed. However, it has been reported that pemetrexed may cause fatigue. Therefore, patients should be cautioned against driving or operating machines if this event occurs.

INSTRUCTION FOR USE

Preparation for Intravenous Infusion Administration

- Use aseptic technique during the reconstitution and further dilution of Pemetrexed for intravenous infusion administration.
- Calculate the dose of Pemetrexed and determine the number of vials needed. Vials contain either 100mg or 500mg of Pemetrexed. The vials contain an excess of Pemetrexed to facilitate delivery of label amount.
- Reconstitute each 100mg vial with 4.2mL of 0.9% Sodium Chloride injection (Preservative free). Reconstitute each 500mg vial with 20mL of 0.9% Sodium chloride injection (preservative free). Reconstitution of either size vial gives a solution containing 5mg/mL Pemetrexed. Gently swirl each vial until the powder is completely dissolved. The resulting solution is clear and range in color from colorless to yellow or green- yellow without adversely affecting product quality. The pH of the reconstituted Pemetrexed solution is between 6.6 and 7.8. Further Dilution is required.
- Parenteral drug products should be inspected visually for particulate matter and discoloration prior to administration, whenever solution and container permit. If particulate matter is observed, do not administer.
- An appropriate quality of the reconstituted Pemetrexed solution must be further diluted into a solution of 0.9% Sodium Chloride Injection (Preservative free), so that the total volume of solution is 100mL. Pemetrexed is administered as an intravenous infusion over 10 minutes.
- Chemical and physical stability of reconstituted and infusion solutions of pemetrexed were demonstrated for up to 24 hours following initial reconstitution, when stored refrigerated. When prepared as directed reconstitution and infusion solutions of Pemetrexed contain no antimicrobial preservatives. Discard any unused portion.

Reconstitution and further dilution prior to intravenous infusion is only recommended with 9% Sodium Chloride Injection (Preservative Free). Pemetrexed is physically incompatible with diluent containing calcium, including Lactated Ringer's Injection, USP and Ringer's injection, USP and therefore these should not be used. Co-administration of Pemetrexed with other drugs and diluents has not been studied, and therefore is not recommended. Pemetrexed is compatible with standard polyvinyl Chloride (PVC) administration sets and intravenous solution bags.

Preparation and administration precaution:

As with other potentially toxic anti-cancer agents, care should be exercised in the handling and preparation of pemetrexed infusion solution. The use of glove is recommended. If a pemetrexed solution contacts the skin, wash the skin immediately and thoroughly with soap and water. If pemetrexed solution contact the mucous membranes, flush thoroughly with water. Pemetrexed is not a vesicant. There is not a specific antidote for extravasation of pemetrexed. There have been few reported cases of pemetrexed extravasation, which were not assessed as serious by the investigator. Extravasation should be managed by local standard practice as with another non-vesicant.

STORAGE CONDITION

Store below 30°C. Protect from light and moisture.

Reconstituted and infusion solutions:

When prepared as directed, reconstituted and infusion solutions of Pemetrexed contain no antimicrobial preservatives. Chemical and physical in-use stability of reconstituted and further diluted infusion solutions of pemetrexed were demonstrated for 24 hours refrigerated temperature (2-8°C).

From a microbiological point of view, the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user and would not be longer than 24 hours at 2°C to 8°C.

SHELF LIFE

Unopened vial: 2 years.

PRESENTATIONS

10ml type 1 clear tubular glass vial was packed in carton together with package insert.

MANUFACTURED BY:

Zulat Pharmacy Sdn Bhd

23 & 23A, Jalan Bandar 3, Taman Melawati,
53100 Kuala Lumpur, Wilayah Persekutuan Kuala Lumpur.

PRODUCT REGISTRATION HOLDER:

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DATE OF REVISION

3/05/2023